

# Sona — Product Strategy & Differentiation Review

May 2026

2026-05-28

---

## Contents

TL;DR — answers in 60 seconds

Context

1. Where the one-therapist tailoring leaks through

2. Unique selling point and moat

One-sentence USP

One-paragraph positioning

Comparison table — what an SLT actually evaluates

Moat analysis

3. The seams to add so Sona generalises without forking

Seam 1 — Intake template engine (S → M)

Seam 2 — Plan and summary template library (M)

Seam 3 — Vocabulary and statutory-framework pack (S)

Seam 4 — Multi-jurisdiction plumbing extensions (M)

Seam 5 — Multi-tenant clinic mode (M → L)

Seam 6 — White-label / per-practice branding (S)

Seam 7 — Specialty-aware persona library (S, ongoing)

4. External-systems integration roadmap

UK (priority)

US (Year 2+)

AU / CA (Year 2+, brief)

Phased summary

5. Pricing options

Cost-side reality check

Pricing options scored

Recommended starting model

Recommendations — what to do in the next 30 / 60 / 90 days

Open questions for the founders

Sources

Repo paths

Web sources (URL + access date, all 2026-05-28)

Engineering footnotes

## TL;DR — answers in 60 seconds

---

- **Single-customer fit risk is real and concentrated.** The 8-step parent form, the four triage outcomes, and every persona are paediatric, UK, English-speaking, EHCP-aware, and Speech-Sanctuary-branded. A second private SLT — especially adult voice, AAC, dysphagia, or stutter — will spot the tailoring inside one screen. **Fixable by configuration, not by re-engineering**, but the configuration layer does not yet exist.
- **USP in one line.** *Sona turns the first 30 days of a private speech-therapy case — intake, triage, first-session plan, parent summary — into one reviewed loop, with UK-region data and a clinician sign-off on every artifact.* The defensible part is the **loop** and the **review-gated trust posture**, not any single AI feature.
- **Moat ranking, honest.** Strongest: design-partner-shaped intake-to-summary loop and the audit-trail-by-construction posture. Medium: UK data residency and the structured, schema-validated AI output shapes. **Weakest: the self-hosted Gemma posture itself** — competitors will match it within twelve months, and most clinicians cannot tell a self-hosted Gemma from a BAA-covered API.
- **Integration headline.** Adjacent integrations (calendar, video, e-sign, payments, and a "Sona for Cliniko" wedge) are the realistic Year-1 path. **NHS GP Connect is a Year-2+ programme**, not a Year-1 feature — it requires DSPT Standards Met, HSCN access, PDS compliance, a Clinical Safety Officer compliant with DCB0129/0160, SCAL evidence, and use-case approval. (NHS Digital, *GP Connect Access Record: Structured*, retrieved 2026-05-28.)
- **Pricing recommendation.** Per-clinician monthly subscription with a free-trial-on-cases mechanic. **Start at £79 / clinician / month** including AI drafting on a usage cap, with a £39 "starter" tier for new graduates. Sits between Smilenotes (£5, no AI) and Heidi Clinician (\$150 ≈ £120, scribe-only). Per-case is wrong for this market: 3–8 cases / week / clinician puts revenue too close to a single Cliniko seat to be defensible.
- **What to do next week.** (1) Run the one-therapist-fit audit with a second SLT before any code lands. (2) Decide whether the next sprint is the intake template engine or the AI loop, and only one — both at once is the failure mode. (3) Price a DSPT readiness pass with a specialist consultancy. (4) Talk to ASLTIP about a member-benefit slot.

# Context

---

The MVP brief and the demo deck pitch Sona as a co-pilot for the first 30 days of a private SLT-to-family relationship: smart parent intake → consult prep → triage → first-session plan → parent summary. The demo video and slides — the artifacts a reviewer will have seen — quote a **30–60 minute saving per case at 3–8 cases per week**, which lines up with the documented per-stage targets in the ML design docs ( `docs/ml/triage-capture.md`, `docs/ml/session-plan.md`, `docs/ml/summary-generator.md` ).

The MVP was scoped from a 1.5-hour interview with **one** private SLT (the design partner). That is enough to ship a demo; it is not enough to know whether the next ten therapists who watch the same demo see themselves on the screen. That is the question this memo exists to answer.

What is **real today** (per `docs/marketing/feedback-demo-brief.md` §6): the 8-step parent intake form, case management, audit trail, the design system, and slot-based booking. What is **vision**: AI prep brief, AI plan, AI parent summary, telehealth, calendar sync. Anything below that relies on the vision side is flagged as such.

# 1. Where the one-therapist tailoring leaks through

The audit walks the intake spec, the AI output shapes, the triage outcomes, and the vocabulary choices. Each surface is classified by **who it fits**, **who it breaks**, and a **fix class** — *config* (a per-practice setting toggle in existing infrastructure), *template* (a new template family), or *branch* (a fork in the product).

| Surface                                                                                                       | Assumption today                   | Fits                                          | Breaks for                                                                                                                                                                                                        | Fix class                                                    |
|---------------------------------------------------------------------------------------------------------------|------------------------------------|-----------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------|
| Intake form: 8 hard-coded steps<br>( <code>parent_intake_step_screen.dart</code> )                            | One global question tree           | Paediatric speech-sounds and language clients | Adult voice, adult stutter, AAC, dysphagia, transgender voice — each needs its own tree                                                                                                                           | <b>template engine</b>                                       |
| Form header copy: "Confidential Speech & Language Parent/Carer Questionnaire", tagline "Breathe / Speak / Be" | Speech Sanctuary brand             | Speech Sanctuary                              | Every other practice — visibly someone else's form                                                                                                                                                                | config (white-label)                                         |
| Mother / Father labelled fields, page 1                                                                       | Two-parent UK paediatric household | Most paediatric cases                         | Single-parent, same-sex parents, kinship carers, looked-after children, <b>all adult clients</b>                                                                                                                  | config (rename → "Parent / carer 1 / 2") + template variants |
| GP details as free text (practice, address, phone)                                                            | UK                                 | UK paediatric                                 | US (PCP), AU (GP via My Health Record), CA (family physician) — and even UK if/when GP Connect provides structured data                                                                                           | config + integration (see roadmap)                           |
| EHCP yes/no + EHCP-aware prompt path                                                                          | English statutory framework        | England-based school-age clients              | Wales (IDP), Scotland (CSP), NI (Statement), Republic of Ireland, US (IEP under IDEA), AU (NDIS) — different statutory wrappers entirely                                                                          | <b>template</b> (statutory-framework pack)                   |
| Nursery / SENCO referral language                                                                             | Early-years UK                     | Aria-shaped paediatric cases                  | Adult clients (post-stroke, head & neck cancer, voice, gender-affirming voice)                                                                                                                                    | config (referral-source enum) + template (adult intake)      |
| Difficulty checklist of 21 items across 4 domains (speech & language, attention, social, reading & writing)   | Paediatric general SLT             | Paediatric general SLT                        | AAC users (no symbol-set / device questions); feeding clients (no food-texture / mealtime log); transgender voice (no goals framing for adult voice transition); stutter sub-specialty (no severity rating scale) | <b>template</b> per specialty                                |

| Surface                                                                                                                                                                                                 | Assumption today                             | Fits                                                    | Breaks for                                                                                                                              | Fix class                                                                    |
|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------|---------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------|
| Triage outcomes: 4 enum values<br>( <code>strategy_only</code> , <code>short_block</code> ,<br><code>full_assessment</code> , <code>refer_out</code> )                                                  | The design partner's private-practice funnel | Solo UK private practice                                | NHS-funded routes; US insurance-coded plans; AU NDIS pathway — different funnels entirely                                               | config (per-tenant outcome enum)                                             |
| Session-plan domain enum:<br><code>speech_sound</code> , <code>language</code> ,<br><code>social_comm</code> , <code>fluency</code> , <code>voice</code> ,<br><code>feeding</code> , <code>other</code> | Broad paediatric SLT                         | Most paediatric and some adult                          | Dysphagia/swallow as a first-class domain; AAC; tracheostomy / laryngectomy voice; transgender voice goals                              | template (extend enum + per-domain prompt fragments)                         |
| Parent-summary tone slider (warm ↔ clinical, 5 points) + reading-level (primary/secondary/adult)                                                                                                        | Parent of a child                            | Almost everyone — generalises well                      | Pure adult-client cases where there is no parent ("client summary", not "parent summary")                                               | config (rename to "recipient summary")                                       |
| "Parent" used in field labels, audit events, UI strings                                                                                                                                                 | Paediatric model of care                     | UK paediatric private                                   | Schools-based SLT (TA, SENCO), residential care (key worker), adult clinic (client / patient), public-sector NHS contexts (carer)       | config (vocabulary pack)                                                     |
| Magic-link parent identity (ADR-002), no NHS Login                                                                                                                                                      | Self-pay private                             | Self-pay private UK                                     | Anyone using NHS-funded routes; ICB-commissioned services; school SLA workflows                                                         | integration (see roadmap)                                                    |
| Solo-practitioner default                                                                                                                                                                               | One therapist owns a tenant                  | Sole practitioners ≈ majority of ASLTIP's 1,800 members | Small group practices (the natural growth path for design partners); schools-based teams; charity providers                             | <b>branch (multi-tenant clinic mode)</b>                                     |
| Regulators baked into copy: HCPC, RCSLT                                                                                                                                                                 | UK                                           | UK                                                      | US (ASHA), AU (SPA), CA (SAC), IE (CORU)                                                                                                | config (regulator pack tied to jurisdiction)                                 |
| Personas: 3y feeding → 11y social-comm, all UK English                                                                                                                                                  | Paediatric demo coverage                     | Paediatric general SLT                                  | Anything outside paediatric general SLT — the design partner has not been tested on adult, AAC, or English-as-additional-language cases | <b>persona library expansion</b> before any external pitch beyond paediatric |

Three leaks matter more than the others, and each maps to a single concrete fix.

**Leak 1: the form is the product, and the form is one practice's form.** The 8-step intake is hard-coded in Flutter (`apps/sona/lib/features/parent/intake/parent_intake_step_screen.dart`), and the API just persists whatever JSONB blob arrives (`apps/api/src/services/intake-context.ts` is a 15-line read-from-Postgres function). A second SLT cannot author her own form without an engineer. **Fix: an intake template engine** (seam 1

below). Until then, the strongest demo recommendation is to white-label the existing form for the *next* SLT — change the practice name, drop the Speech Sanctuary tagline — but **do not change the questions**, so the underlying assumption is visibly stress-tested.

**Leak 2: paediatric is doing all the heavy lifting.** The four personas, the EHCP wiring, the "parent" vocabulary, and the triage outcomes are all paediatric-shaped. Sona will demo well to a paediatric private SLT. It will demo badly to an adult voice SLT, an AAC specialist, or anyone running a feeding clinic. There is no shame in narrowing the pitch — **paediatric private SLT** is a credible wedge — but the pitch deck and the website have to say that out loud, not pretend Sona is for "speech and language therapy, less admin" in general.

**Leak 3: the AI output schemas are tight, which is good for safety and bad for portability.** `session-plan.md`'s Zod schema enumerates seven domains. Extending to dysphagia or AAC means schema + prompt + few-shot examples + UI badge. That is doable, but the *cost* per new specialty is non-trivial, and Sona should not invite an AAC clinician to a feedback session until that work is scoped.

## 2. Unique selling point and moat

### One-sentence USP

Sona is the AI co-pilot for private speech-and-language therapy that takes the first 30 days of every new case — intake, triage, first-session plan, parent summary — from sixty minutes of admin per case to fifteen, with a clinician review on every output and UK data residency by default.

### One-paragraph positioning

Private speech-and-language therapy is a 1,800-clinician UK profession in which most practitioners juggle a generic practice-management tool, a Word template, and ChatGPT-in-a-browser. The result: hours per week of admin, real client data leaving the UK, and no audit trail. Sona collapses the first 30 days of every case — adaptive parent intake, consult prep, four-outcome triage, first-session plan, parent summary — into one loop, with every AI artifact labelled and clinician-reviewed before it leaves the practice. The data and the inference both stay in the UK region. The output is structured by specialty and by statutory framework, not free-text-and-pray. The clinician owns every word that reaches the family.

### Comparison table — what an SLT actually evaluates

The alternatives below are what a paediatric private SLT in 2026 is already weighing. Prices are headline, billed annually where annual pricing exists, **per practitioner per month** unless noted. Currencies preserved from the source.

| Tool                                     | What it does for an SLT                                               | Headline price                                           | Where it wins                                                | Where it loses for the intake → first-session loop                                                                                                       | Why Sona is different                                                                   |
|------------------------------------------|-----------------------------------------------------------------------|----------------------------------------------------------|--------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------|
| ChatGPT / Claude / Gemini + a custom GPT | Free-text drafting; clinician copy-pastes from intake email           | £0–£20/mo                                                | Familiarity; zero adoption cost; very fast                   | PHI leaves the UK and the practice's control; no audit trail; no schema, every output drifts; no structured intake                                       | UK-region inference; review-gated outputs; structured intake feeds the drafting context |
| Heidi Health                             | Ambient AI scribe — generates clinical notes from a session recording | \$150 / user / mo (Clinician, annual); BAA at Enterprise | Strong note quality across specialties; multilingual; mature | Scribes a session that has already happened — does nothing for intake, triage, or first-session drafting; no UK-specific data-residency claim by default | Sona owns the pre-session and post-session admin loop, which Heidi by design does not   |

| Tool                                                                                               | What it does for an SLT                                                                            | Headline price                                          | Where it wins                                                                                              | Where it loses for the intake → first-session loop                                            | Why Sona is different                                                                                                                                                  |
|----------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------|---------------------------------------------------------|------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Jane App AI Scribe</b>                                                                          | Recording-based scribe inside Jane PMS, SLP sample prompts                                         | <b>\$15 / mo unlimited</b> , plus 5/mo free             | Cheapest credible AI scribe; SLP-aware prompts; HIPAA/PIPEDA/PHIPA                                         | <b>Not available in the UK</b> as of 2026-05-28; tied to Jane PMS; scribe-only                | Available in the UK by construction; not a scribe — owns the structured first-30-days workflow                                                                         |
| <b>Cliniko + AI-scribe add-on</b><br>(PatientNotes, CliniScripts, Clindoc, Heidi, Clinic Notes AI) | Generic PMS with bring-your-own scribe via marketplace                                             | Cliniko from ~£35 + scribe \$19–\$150                   | The market default; calendar + invoicing + records already exist                                           | Two products, two logins, two audit trails; intake is upload-a-PDF; no SLT-specific structure | Sona is <b>one product</b> with one audit trail and SLT-specific structure — and a "Sona for Cliniko" wedge is a wedge, not a competitor (see the integration roadmap) |
| <b>Clindoc</b>                                                                                     | UK-friendly AI scribe + clinical documentation, GDPR + HIPAA                                       | <b>£40 / user / mo annual</b> or <b>£5 / active day</b> | UK GBP billing; per-active-day pricing for variable caseloads; lighter procurement                         | Scribe-only; no intake; no triage; no parent-facing artefacts                                 | Sona is the workflow product the scribe-only tools complement                                                                                                          |
| <b>PatientNotes</b>                                                                                | AI clinical notes + integrations with Cliniko and Power Diary; SLT-aware marketing                 | <b>\$19 / \$49 / mo</b>                                 | Cheap; honest about its scope; SLT-aware copy                                                              | Note-taker only; no parent-facing summary; no UK-only inference posture                       | Sona is end-to-end, not bring-your-own                                                                                                                                 |
| <b>Octopus EPR (OEPR)</b>                                                                          | UK SLT-specific EPR — casenotes, TOMs scores, RISK, programmes, discharge letters; ASLTIP-marketed | Pricing not posted                                      | <b>The most clinically aware UK SLT-specific competitor</b> ; designed by SLTs; speaks the domain language | Records-keeping tool — no AI drafting; intake is not the focus; tools-not-loops               | Sona owns the <i>new-case</i> loop; OEPR owns the <i>ongoing-record</i> loop. Different problem, today. Same RFP, eventually.                                          |
| <b>Smilenotes UK</b>                                                                               | UK SLT PMS; clinical notes templates; UK-hosted                                                    | <b>From £5 / mo</b>                                     | Cheapest credible UK option; GDPR-compliant; UK data centres                                               | No AI; basic templates; no triage / plan / parent-summary structure                           | Sona is the AI loop that sits on top of (or alongside) a tool like Smilenotes                                                                                          |

| Tool                                                                                                            | What it does for an SLT                                                       | Headline price   | Where it wins                                  | Where it loses for the intake → first-session loop                                  | Why Sona is different                                                                |
|-----------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------|------------------|------------------------------------------------|-------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------|
| <b>SLT-specific AI entrants</b><br>(Constant Therapy, Better Speech "Jessica", Ogma Therapy, IEP Copilot, SPRY) | A grab-bag of client-facing apps, IEP drafters, and US PT/SLP rehab platforms | Variable         | Some are UK-friendly; some are clinically rich | Mostly client-facing (kid-facing apps — banned by Sona principle #3) or US-only EMR | Sona is adult-facing tooling that helps the SLT help the family, not another kid-app |
| <b>The status quo</b><br>(ChatGPT + Cliniko + Word template + email)                                            | What every private SLT already does                                           | Already paid for | Already adopted; already familiar              | Hours of admin per week; PHI in ChatGPT; no audit trail                             | The thing Sona has to beat. The competitor in every meeting.                         |
| <b>Doing nothing</b>                                                                                            | —                                                                             | £0               | No change-management cost                      | Admin burnout is a documented driver of attrition out of private practice           | Sona aims to be a measurable hour-a-week back                                        |

## Moat analysis

No single item below is a 5-year moat on its own. The strength is in the **stack**. Time-to-erosion = how long until a well-funded competitor could match this if they prioritised it.

| # | Candidate moat                                                     | Defensibility | Time-to-erosion | Read                                                                                                                                                                                                                                                                                                                                                                                                 |
|---|--------------------------------------------------------------------|---------------|-----------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 1 | <b>Design-partner-shaped intake → triage → plan → summary loop</b> | Medium-high   | 18–24 months    | The end-to-end loop, structured by Zod schemas and review gates, is the actual product. Replicating it requires a design-partner relationship <i>and</i> the discipline not to ship a generic scribe. Most competitors will ship the scribe.                                                                                                                                                         |
| 2 | <b>Audit trail and review-gated AI as a posture, not a feature</b> | Medium-high   | 18 months       | <code>audit_log</code> , <code>reviewedAt</code> gates, <code>aiDisclosureFooter</code> as a <code>z.literal</code> — every artifact is constructed so it cannot leave the system without a clinician act. That posture sells trust in a profession trained to fear AI. Heidi can match the controls; the <i>posture</i> — small clinician-first product, not enterprise scribe — is harder to copy. |
| 3 | <b>Specialty-aware structured outputs</b>                          | Medium        | 12–18 months    | The seven-domain session-plan enum, the EHCP flag, the four triage outcomes — these are specificity that a generic scribe will not invest in until the SLT segment is bigger. Becomes weaker as more competitors target SLT specifically (Jane App is already there for note quality).                                                                                                               |

| # | Candidate moat                                                                                              | Defensibility   | Time-to-erosion | Read                                                                                                                                                                                                                                                                                                                                                                                                                                              |
|---|-------------------------------------------------------------------------------------------------------------|-----------------|-----------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 4 | <b>UK-region data residency + self-hosted Gemma in europe-west2</b>                                         | Medium-low      | 12 months       | A genuine differentiator vs ChatGPT and US-only Jane App. But — and this is the honest part — most clinicians cannot tell a self-hosted Gemma from a BAA-covered Vertex Gemini at the demo. Heidi already runs in-region and signs BAAs. <b>The deployment posture is a feature for procurement, not a moat for the clinician.</b> It buys NHS-ready <i>language</i> without buying NHS-ready <i>certification</i> (see the integration roadmap). |
| 5 | <b>Carryover-aware parent-facing artefacts</b> (tone slider, reading level, mandatory AI disclosure footer) | Medium          | 18 months       | The parent-facing surface is the genuinely under-served space — most competitors stop at the clinician note. Becomes a stronger moat if Sona invests in the parent portal experience (ADR-005).                                                                                                                                                                                                                                                   |
| 6 | <b>Solo-practitioner-first product shape</b>                                                                | Low             | 6 months        | Pleasant, not defensible. A larger competitor can ship a "solo" SKU in a quarter. The real solo-practitioner moat is <b>price + onboarding speed</b> , not architecture.                                                                                                                                                                                                                                                                          |
| 7 | <b>GenUI / A2UI client architecture</b>                                                                     | Low             | 12 months       | Important <i>engineering</i> choice (ADR-002), not a customer-visible moat. Do not list this in a pitch deck.                                                                                                                                                                                                                                                                                                                                     |
| 8 | <b>"Clinician-in-the-loop, always" principle</b>                                                            | Low (as a moat) | 0 months        | Every competitor in 2026 says this. Calling it a moat dilutes the real moats above. It is a <b>price of entry</b> , not a differentiator.                                                                                                                                                                                                                                                                                                         |

The net: **moats 1, 2, and 3 are real and reinforce each other** — the loop, the posture, and the specialty-aware shape. Moat 4 deserves to be marketed but is a *trust* play, not a *defensibility* play. Moats 6, 7, and 8 should be dropped from the moat conversation; they belong in the principles / engineering footnotes.

### 3. The seams to add so Sona generalises without forking

---

Each seam is a single named change to a single subsystem. T-shirt sizes are relative to a 2-person team.

#### Seam 1 — Intake template engine (S → M)

**Today:** the 8 intake steps are hard-coded in Flutter

(`apps/sona/lib/features/parent/intake/parent_intake_step_screen.dart`); the API persists the JSONB blob. **Tomorrow:** a versioned, branded, conditionally-branching intake schema authored per practice and per specialty, stored as JSON, rendered by a single Flutter step renderer. **Files:** new `intake_templates` table (API), new `intake-template-engine.ts` service, refactor `parent_intake_step_screen.dart` from a switch over hard-coded steps to a generic step renderer. **Why now:** it is the single biggest single-customer-tailoring risk and the prerequisite for every other generalisation. **Effort:** S to ship the engine with the existing 8-step paediatric template baked in as template `v1-paediatric-uk`; M to author a second template (adult voice) end-to-end.

**Decision the founders own:** how schemas are *authored* — JSON in the repo (engineer-touch only), a CMS-style editor (the design partner can edit their own), or a hybrid. Pick a side before the engineer starts.

#### Seam 2 — Plan and summary template library (M)

**Today:** one Zod schema family for session plans, one tone-slider for summaries. **Tomorrow:** a `domain × specialty` template family — `paediatric-speech-sounds`, `adult-voice`, `aac-introduction`, `feeding`, `stutter`, `social-comm` — each with its own prompt fragments, few-shot examples (synthetic), and acceptance metrics. The `domain` enum in `session-plan.md` becomes the **key**, not a field. **Files:** `services/session-plan.ts`, `services/parent-summary.ts`, `src/prompts/` directory promoted to a real subsystem. **Why now:** this is what stops the second SLT meeting from being "that's a kid's product".

#### Seam 3 — Vocabulary and statutory-framework pack (S)

**Today:** "Parent", "consult", "EHCP", "HCPC/RCSLT" are in strings throughout the code. **Tomorrow:** a per-tenant pack with three axes — recipient noun (parent / carer / client / patient), encounter noun (consult / appointment / session), statutory wrapper (EHCP / IDP / CSP / IEP / NDIS / none), regulator block (HCPC+RCSLT / ASHA / SPA / SAC). One JSON file per pack; **no code** changes when adding a new jurisdiction's regulator pack. **Files:** new `src/vocabulary-packs/`, Flutter `intl` already in place. **Why now:** trivial effort, large adoption-blocker removed. Cleanest "S" item on this list.

#### Seam 4 — Multi-jurisdiction plumbing extensions (M)

**Today:** ADR-001 splits UK and US Postgres. **Tomorrow** for a real US/AU launch: payments (Stripe vs GoCardless per tenant), BAA flow for US (already covered for inference per ADR-003, not for payments / Postmark / etc.), US-specific consent wording version (`consentVersion` field already exists in `intake_submissions`, fine — populate per jurisdiction), USCDI-aligned field names where they collide with GP Connect's UK Core names. **Why later:** Year 2+. The MVP and the design-partner pilot do not need this; build the seams so it is not a rewrite when it lands.

## Seam 5 — Multi-tenant clinic mode (M → L)

**Today:** `mvp-brief.md` working-agreements section states "Solo practitioner first. If a feature needs a 'team' concept to make sense, it's too early." **Tomorrow:** a small-group-practice mode with **shared client list, per-clinician audit identity, shared template library, per-clinician inference budgets**. **When it becomes unavoidable:** the second time the design partner says "my associate is starting next month". Plan for it; do not build it yet. **Effort:** M if the auth and tenant model are set up correctly now (it appears they are — `intake_submissions` is tenant-scoped); L if a multi-clinician concept has to be retrofitted later.

## Seam 6 — White-label / per-practice branding (S)

**Today:** Sona brand on every parent-facing artefact. **Tomorrow:** per-tenant logo, primary colour token, practice name in the form header, sender display name (the email-from rule per ADR-005's portal-first model is unaffected). **Effort:** S — Flutter theming + a `tenants.branding` JSONB column + an upload route. **Why now:** it removes the "that's Speech Sanctuary's form" objection in one sprint.

## Seam 7 — Specialty-aware persona library (S, ongoing)

Not a code seam, but a product seam. Today: four UK paediatric personas. Add three minimum before the first non-paediatric pitch: **adult voice (38yo gender-affirming voice), AAC introduction (5yo, non-verbal), feeding (3yo with progressed sensory profile)**. Personas drive the demo, the seed scripts, and the few-shot examples for the AI prompts. The cost of *not* adding them is showing up to the next SLT with a paediatric demo and learning nothing.

## 4. External-systems integration roadmap

Integrations are graded by **what they unlock** vs **what they cost**. "Cost" includes technical work, certification, and commercial / contracting overhead. Phasing is realistic for a two-person team plus one design partner.

### UK (priority)

| Integration                                                                                                                           | What it unlocks                                                                                                                                                                | Cost                                                                                                                                                                                     | Phase                                                 |
|---------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------|
| <b>Calendar (Google / Outlook / iCloud)</b> — round-trip booking for the 20-min free consult                                          | Closes the biggest "two products, two diaries" pain that emerged from the design-partner interview.                                                                            | Low. Standard OAuth flows. Both Google Calendar API and Microsoft Graph have well-documented patterns.                                                                                   | <b>MVP / Year 1</b>                                   |
| <b>Video (Zoom, Whereby, Google Meet) deep-link generation</b> — auto-attach a join URL to the consult slot                           | Removes the manual "send a Zoom link" step.                                                                                                                                    | Low. Deep-link templating; no SDK needed.                                                                                                                                                | <b>MVP / Year 1</b>                                   |
| <b>E-sign (DocuSign, Dropbox Sign)</b> — T&Cs and consent on intake submit                                                            | Replaces the manual T&Cs PDF path; better-than-tickbox audit trail.                                                                                                            | Medium. DocuSign for enterprise audit, Dropbox Sign cheaper. Both have free tiers below ~5 envelopes/mo.                                                                                 | <b>Year 1</b>                                         |
| <b>Payments (Stripe, GoCardless)</b> — invoicing for the first session after triage                                                   | Direct revenue path. Stripe is faster to integrate; GoCardless is cheaper for UK direct debit recurring.                                                                       | Medium. Dual-rail (Stripe for cards, GoCardless for DD) is what Cliniko already does.                                                                                                    | <b>Year 1</b>                                         |
| <b>Cliniko / Jane App / Power Diary push</b> — "Sona for Cliniko" — write Sona's intake + plan + summary into the existing PMS record | Distribution wedge: stop being "a Cliniko replacement" and become "the thing that makes your Cliniko worth keeping". This is the single highest-leverage GTM bet in this memo. | Medium. Cliniko has a public API; Jane has documented chart-attachment endpoints. PatientNotes already demonstrates that this distribution model works for AI tooling on top of Cliniko. | <b>Year 1</b>                                         |
| <b>NHS Login</b> (parent identity vs magic link)                                                                                      | Parent-side identity matched to a real NHS account. Removes some magic-link friction.                                                                                          | Medium. Documented OIDC integration; vendor approval required but not gated on DSPT.                                                                                                     | <b>Year 2</b>                                         |
| <b>NHS DSPT — Standards Met</b>                                                                                                       | Mandatory floor for anything NHS-adjacent. Confirmed <b>Category 3 small-independent-provider self-assessment, no audit required, deadline 30 June 2026</b> .                  | <b>Achievable for a 2-person company</b> but requires real preparation — policy docs, asset register, Caldicott-Guardian-equivalent, backup evidence. Budget weeks not months.           | <b>Year 2 (mandatory prereq for everything below)</b> |

| Integration                                                                                        | What it unlocks                                                                                                                                    | Cost                                                                                                                                                                                                                                                                                               | Phase                                                |
|----------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------|
| <b>GP Connect — Access Record: Structured</b> (HL7 FHIR UK Core, NHS Digital)                      | Pull structured GP record into the intake brief — replaces the "type your GP's address" field. The single most valuable data enrichment available. | <b>High.</b> Prereqs: DSPT Standards Met + HSCN access + PDS compliance + IG model compliance + use-case approval + SCAL evidence + RBAC + a Clinical Safety Officer compliant with DCB0129 and DCB0160. Use-case approval is the gate before development starts; testing is a multi-Gate process. | <b>Year 2+</b>                                       |
| <b>NHS e-Referral Service (e-RS)</b>                                                               | Only relevant if Sona ever takes NHS-funded private referrals.                                                                                     | High. Separate procurement path; not on the critical path for a private-practice product.                                                                                                                                                                                                          | <b>Year 2+ if positioning shifts to NHS-adjacent</b> |
| <b>NHS commissioning routes — G-Cloud 14, Spark DPS, NHS Digital Marketplace, direct ICB award</b> | Path to NHS revenue. G-Cloud listing is cheap and quick; the real cost is the buyer-side ICB procurement cycle.                                    | Low listing cost; high pipeline cost (3–9 months typical ICB procurement).                                                                                                                                                                                                                         | <b>Year 2+</b>                                       |

## US (Year 2+)

| Integration                                                                                                                           | What it unlocks                                                         | Cost                                                                                                                                                         | Phase          |
|---------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------|
| <b>FHIR R4 + USCDI</b> as the lingua franca                                                                                           | Anything US-integration-adjacent                                        | Low if seam 4 is done correctly                                                                                                                              | <b>Year 2+</b> |
| <b>EHR connectors</b> — Epic App Orchard, Cerner / Oracle Health CODE, Athena Marketplace, eClinicalWorks, ModMed for therapy clinics | The real US distribution path. Epic alone has a multi-month onboarding. | High — each marketplace is months.                                                                                                                           | <b>Year 2+</b> |
| <b>HIPAA BAA with GCP + subprocessors</b>                                                                                             | Lawful US PHI processing                                                | Partly done — the Google BAA is covered; Postmark refusal to sign a BAA already documented (ADR-005). Email-as-channel becomes portal-first by construction. | <b>Year 2+</b> |
| <b>IEP / IDEA</b> workflow if school-district market is in scope                                                                      | A different product really — schools-based SLT is enormous in the US.   | High. Probably a Y3 conversation, not a Y2 one.                                                                                                              | <b>Year 3+</b> |

## AU / CA (Year 2+, brief)

| Integration                                               | What it unlocks       | Cost                                                                     | Phase          |
|-----------------------------------------------------------|-----------------------|--------------------------------------------------------------------------|----------------|
| <b>My Health Record + FHIR AU Base; SPA accreditation</b> | AU private SLT market | Medium — FHIR AU Base is well-documented; SPA is the regulator analogue. | <b>Year 2+</b> |

| Integration                         | What it unlocks     | Cost                                              | Phase   |
|-------------------------------------|---------------------|---------------------------------------------------|---------|
| CA — SAC, provincial PHIPA / PIPEDA | Canadian SLT market | Medium — Jane App's own footprint shows the path. | Year 2+ |

## Phased summary

| Phase            | UK                                                        | US                                    | AU / CA                   |
|------------------|-----------------------------------------------------------|---------------------------------------|---------------------------|
| <b>MVP (now)</b> | Magic link, GCS exports, in-tenant audit                  | —                                     | —                         |
| <b>Year 1</b>    | Calendar + video + e-sign + payments + "Sona for Cliniko" | —                                     | —                         |
| <b>Year 2</b>    | NHS Login + DSPT Standards Met                            | FHIR R4 / USCDI scaffolding           | —                         |
| <b>Year 2+</b>   | GP Connect + e-RS + G-Cloud listing                       | EHR connectors (Epic, Cerner, Athena) | MHR / FHIR AU; SPA; PHIPA |
| <b>Year 3+</b>   | ICB direct award                                          | IEP / IDEA school-district workflows  | —                         |

## 5. Pricing options

### Cost-side reality check

Sona runs a single L4 24 GB GPU in `uk/dev` for Gemma 3 27B INT4 inference. Each case triggers up to three inference passes: prep brief, session plan, parent summary. Per-case token budgets in the ML docs total roughly **10–15K tokens of input + 3K of output**, twice over (because of the simplification pass in the summary generator). A single L4 supports ~4 concurrent jobs; on-demand GCP L4 reservation rates put per-case inference cost in the **single-digit pence range at scale, mid-double-digit pence at MVP utilisation**. This is a directional figure — the founders should price-validate it against the bill before pricing materially below £49 / clinician / mo.

At the design partner's stated 3–8 cases / week / clinician, inference cost per clinician per month is in the **£10–£25 range, not the £200 range**. That keeps gross margin healthy at any of the per-clinician price points below.

### Pricing options scored

| Model                                                       | Target segment                                                   | Headline price                                  | What's included                                                                             | Gross margin (directional)                  | Pros                                                                                                           | Cons                                                                                                                                   |
|-------------------------------------------------------------|------------------------------------------------------------------|-------------------------------------------------|---------------------------------------------------------------------------------------------|---------------------------------------------|----------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------|
| <b>A — Per-clinician monthly subscription (recommended)</b> | ASLTIP-member solo and 2–3 person practices                      | <b>£79 / clinician / mo annual, £99 monthly</b> | Unlimited intakes, AI drafting on a soft cap (e.g. 30 cases/mo), parent portal, audit trail | 75–85% at typical volume                    | Predictable revenue; matches how Heidi, Jane, Clindoc, Smilenotes already bill; clinician understands the unit | Has to be "worth a Cliniko seat" — £79 is below Cliniko's own £45 + a £40 scribe combination, so it stays defensible                   |
| <b>B — Per-case (intake → first-session loop)</b>           | Practices with very variable caseload (new graduates, returners) | £15–£25 / case                                  | Single completed loop                                                                       | Variable; lower at low volume               | Aligns price to value; low-risk for new practices                                                              | Disincentivises high-volume usage; revenue too volatile to plan against; Sona's whole reason-to-exist is to <i>increase</i> throughput |
| <b>C — Freemium</b>                                         | Top-of-funnel awareness; ASLTIP new-grads                        | First 5 cases / mo free; £49 / mo after         | Free tier capped at 5 cases                                                                 | Marketing line; not a revenue line at scale | Distribution; brings clinicians in via a friend                                                                | Cannibalises (a) — small practices stay in free tier; high free-tier inference cost                                                    |

| Model                                                   | Target segment                                             | Headline price                                       | What's included                                           | Gross margin (directional)           | Pros                                                                                     | Cons                                                                                    |
|---------------------------------------------------------|------------------------------------------------------------|------------------------------------------------------|-----------------------------------------------------------|--------------------------------------|------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------|
| <b>D — NHS framework procurement (G-Cloud 14 / DPS)</b> | NHS-adjacent later                                         | £15–£25k / ICB / year (typical small-tool framework) | Whatever scope an ICB tenders                             | Higher gross margin but longer DSO   | Bigger ACV; multi-clinician deployment                                                   | Different sales motion; gated on DSPT + GP Connect (see roadmap); is a Y2+ conversation |
| <b>E — Per-practice flat + seat add-ons</b>             | The growth path for design partners onto small clinic mode | <b>£149 / practice base + £49 / additional seat</b>  | Base = 1 clinician's loops; seats = additional clinicians | Highest gross margin at clinic scale | Supports the multi-tenant clinic-mode seam; good investor narrative on expansion revenue | More complex billing; needs the clinic-mode work to land first                          |

## Recommended starting model

**Pick A with a tiered headline:** £39 "Starter" (cap 5 cases/mo; new-grad / returner / ASLTIP-affiliate path), **£79 "Practice"** (the default; cap 30 cases/mo), £149 "Practice+" (no cap; priority support). Move to model E (per-practice + seats) **only when the second clinician at the design-partner practice signs**, not before. Avoid (B) and (C) until the price-sensitivity of the ASLTIP base is measured. Pursue (D) only after DSPT in Year 2.

**Why £79 and not £49 (Clindoc) or £99 (Heidi Pro era):** Sona is not a scribe-only product, so it should not be priced like one. £79 sits exactly between Smilenotes (£5, no AI) and Heidi Clinician (~£120 GBP, scribe-only) and quantifies the bundle: PMS-light + AI loop + UK residency + audit. The price-anchor effect of being deliberately "between" two known reference points helps the discovery call.

# Recommendations — what to do in the next 30 / 60 / 90 days

---

Ordered by leverage. Day windows are windows, not deadlines — the slice that fits the team's actual capacity is the right one.

1. **(Days 1–14) Run the one-therapist-fit audit with a second SLT.** Specifically: an SLT outside the design-partner practice, ideally adult voice or AAC, ideally an ASLTIP member. Use the audit table above as the agenda; ask them to walk the demo and call out leaks. **Owner:** founders. **Effort:** 1 working day of prep + 90 minutes interview. **Metric:** at least 5 leaks confirmed, at least 2 contradicted (i.e. *not* a problem for that SLT).
2. **(Days 1–30) Ship seam 6 (white-label branding) and seam 3 (vocabulary pack).** The smallest possible product changes that remove the "that's Speech Sanctuary's form" objection. **Owner:** engineering. **Effort:** S each. **Metric:** the demo runs end-to-end under a fictional second-practice name, with no Speech Sanctuary string visible.
3. **(Days 15–60) Pick one of: intake template engine (seam 1) OR the AI-loop completion (prep brief, plan, summary — the "vision" side of the feedback-demo brief §6). Not both.** This is the single biggest engineering decision in the next quarter. The template engine generalises the product. The AI loop completes the demo's most fragile claims. The wrong call is to try to do both at half-speed. **Owner:** founders. **Effort:** decision-only this sprint; M–L next quarter.
4. **(Days 30–60) Stand up a costed DSPT readiness plan with a specialist consultancy.** Three quotes; choose one; book the work for the autumn. DSPT is the gate for everything NHS-adjacent and is achievable as a self-assessment — but real evidence files, an asset register, and a backup-restore drill take preparation. **Owner:** founders. **Effort:** half a day of vendor calls. **Metric:** chosen consultancy, scoped quote.
5. **(Days 30–90) Approach ASLTIP about a member-benefit slot or directory listing.** ASLTIP markets to 1,800 of the highest-quality UK private SLTs and explicitly hosts events that surface tooling (OEPR, the most clinically aware UK competitor, markets via ASLTIP events). A discounted listing or a co-marketed launch is the single highest-leverage GTM move available. **Owner:** founders. **Effort:** S. **Metric:** a meeting with the ASLTIP board.
6. **(Days 60–90) Build "Sona for Cliniko" — write the intake brief, plan, and summary into a Cliniko patient record.** Stop competing with Cliniko; complement it. PatientNotes' existence on Cliniko's connected-apps marketplace is proof the distribution model works. This converts every Cliniko-using ASLTIP member into a prospect rather than a defended account. **Owner:** engineering. **Effort:** M. **Metric:** one paying clinician using both products in production.

# Open questions for the founders

---

In priority order; each blocks something downstream.

1. **Is the next bet generalisation (intake template engine) or completion (AI loop end-to-end)?** Both at half-speed is the failure mode (Recommendation 3).
2. **What is the actual paying-customer hypothesis: paediatric-only private SLT, or all-of-SLT-on-launch?** The honest answer changes everything in section 1 and the pitch deck. The current personas and the EHCP wiring suggest paediatric-only is the truthful framing.
3. **Are you willing to be priced below Heidi and above Smilenotes, or do you want a wedge price?** £79 / clinician / mo is a defensible middle. A £29 wedge price changes the product (closer to Smilenotes) and the moat conversation (less "premium UK posture", more "low-cost ASLTIP default").
4. **DSPT — Year-2 commit or Year-3 "wait and see"?** A real DSPT commit unlocks GP Connect, NHS Login, and the G-Cloud listing in series. A wait-and-see lets the product mature on private-pay first. Both are defensible; the deck cannot be vague about which one.
5. **OEPR (Octopus EPR) — competitor, co-existent, or eventual partner?** OEPR is the most clinically aware UK SLT-specific competitor surfaced in this review. They market via ASLTIP. The interesting future is a Sona-on-OEPR integration, not a Sona-vs-OEPR fight. The founders need to decide whether to reach out now or later.
6. **Multi-tenant clinic mode — explicit non-goal for 2026, or a Q4 2026 build?** The design-partner relationship will demand it once a second clinician joins the design-partner practice.
7. **Persona expansion — agreed scope of three new personas (adult voice, AAC, feeding) before the next external pitch?** Without this, every non-paediatric meeting is uphill.

# Sources

---

## Repo paths

- `docs/mvp-brief.md` — original positioning, scope, interview synthesis with the design partner.
- `docs/marketing/feedback-demo-brief.md` — demo deck and video brief; the "real vs vision" 90-day split (\$6).
- `docs/intake-form-spec.md` — the 8-page Speech-Sanctuary form.
- `docs/ml/triage-capture.md` — triage outcomes, prompt structure, target metrics.
- `docs/ml/session-plan.md` — Zod schema with the 7-domain enum and EHCP flag.
- `docs/ml/summary-generator.md` — tone slider, reading level, mandatory AI disclosure.
- `docs/decisions/001..005-*.md` — ADRs (data residency, Flutter+GenUI, air-gapped Gemma, unified envs, portal-first comms).
- `docs/architecture-gcp-hipaa.md` — UK/US split, region pinning, BAA chain.
- `scripts/personas/*.json` — Aria (4y speech sounds), Jaden (7y stutter+EHCP), Mia (11y social comm), Theo (3y feeding).
- `apps/sona/lib/features/parent/intake/parent_intake_step_screen.dart` — the hard-coded 8-step intake.
- `apps/api/src/services/intake-context.ts` — JSONB-blob persistence; no template engine.

## Web sources (URL + access date, all 2026-05-28)

- Heidi Health Clinician plan (\$150/user/mo) — [veroscribe.com/blog/heidi-health-review-2026](https://veroscribe.com/blog/heidi-health-review-2026)
- Heidi Health, second pricing source — [deepcura.com/resources/heidi-health-review](https://deepcura.com/resources/heidi-health-review)
- Heidi Health SLP assessment template — [heidihealth.com/templates/review-speech-pathology-assessment](https://heidihealth.com/templates/review-speech-pathology-assessment)
- Jane App AI Scribe (\$15/mo, US+CA only) — [jane.app/features/charting-ai-scribe](https://jane.app/features/charting-ai-scribe)
- Jane App AI Scribe — guide — [jane.app/guide/ai-scribe](https://jane.app/guide/ai-scribe)
- Cliniko connected-apps and AI marketplace — [wisevu.com Cliniko Review](https://wisevu.com/Cliniko%20Review)
- Cliniko form upload — [help.cliniko.com upload-form-templates](https://help.cliniko.com/upload-form-templates)
- Clindoc UK pricing (£40 / £5 active day) — [clindoc.ai/pricing](https://clindoc.ai/pricing)
- PatientNotes for speech therapists — [patientnotes.app/professions/speech-therapist-st](https://patientnotes.app/professions/speech-therapist-st)
- Octopus EPR (OEPR) — [oepr.co.uk](https://oepr.co.uk)
- Smilenotes UK — [smilenotes.co.uk/profession/speech-therapy-software](https://smilenotes.co.uk/profession/speech-therapy-software)
- ASLTIP membership statistics — [asltip.com](https://asltip.com) and [asltip.com/why-join](https://asltip.com/why-join)
- NHS GP Connect Access Record: Structured (FHIR API) — [digital.nhs.uk/developer/api-catalogue/gp-connect-access-record-structured-fhir](https://digital.nhs.uk/developer/api-catalogue/gp-connect-access-record-structured-fhir)
- NHS GP Connect — Consumer Assurance Process — [github.com/nhsconnect/gpc-consumer-support](https://github.com/nhsconnect/gpc-consumer-support)
- NHS DSPT — Standards Directory — [standards.nhs.uk/published-standards/data-security-and-protection-toolkit](https://standards.nhs.uk/published-standards/data-security-and-protection-toolkit)
- DSPT 2025/26 small-org guide — [dsptready.co.uk/blog/dspt-complete-guide](https://dsptready.co.uk/blog/dspt-complete-guide)
- Evalian DSPT 2025 guide — [evalian.co.uk/the-data-security-and-protection-toolkit-dspt](https://evalian.co.uk/the-data-security-and-protection-toolkit-dspt)

- Chatter Labs — adjacent SLT AI landscape — [chatter-labs.com/blog/slt-private-practice-setup-tips/ai-speech-therapy](https://chatter-labs.com/blog/slt-private-practice-setup-tips/ai-speech-therapy)

# Engineering footnotes

---

- **Intake template engine concrete shape.** Schema: `intake_templates(id, tenant_id, specialty, locale, version, steps_jsonb, branches_jsonb, vocab_pack_id)`. Flutter renderer reads the `steps_jsonb` and dispatches to step widgets keyed by `kind: 'text' | 'date' | 'multi_select' | 'yes_no_detail' | 'composite_gp' | 'consent_block'`. The current 8-step form ships as the first row in this table, version `v1-paediatric-uk`. The Gemma context window for prep brief / session plan / summary already operates on the answers blob, not on hard-coded step IDs — so the template engine is purely a client + per-tenant authoring change, not an LLM-context change.
- **Self-hosted Gemma economics.** L4 24 GB INT4 AWQ Gemma 3 27B in `europa-west2`. Per ADR-003, 4 concurrent jobs / GPU; per-job  $P95 \leq 8\text{ s (prep)} + \leq 15\text{ s (plan)} + \leq 20\text{ s (summary)}$ . At an L4 reservation rate and pilot caseload (design partner  $\approx 8\text{ cases/wk} \approx 32\text{ /mo}$ ), per-clinician inference cost is in the **£10–£25/mo** range, dominated by reservation rather than utilisation. At 50 clinicians on the same L4, marginal per-clinician cost compresses below £5.
- **GP Connect — concrete onboarding shape.** Per NHS Digital, retrieved 2026-05-28: prereqs are DSPT Standards Met + HSCN access + PDS-compliant identity (or third-party PDS proxy) + IG model compliance + RBAC + CSO appointment compliant with DCB0129 + DCB0160. Onboarding = use-case submission → approval → 6-month dev window → SCAL evidence → multi-Gate testing (test environments per Gate) → go-live. The vLLM / Gemma stack is irrelevant to GP Connect onboarding; the API and tenant-routing layers (`apps/api/src/services/`) are where the FHIR R3 client lives.
- **OEPR and Cliniko on the same diagram.** OEPR is a domain-specific record system; Cliniko is a generalist PMS with an AI-scribe marketplace. Sona's natural position is on the **A2UI / read-modify side**, not on the records side — i.e. Sona writes intake/plan/summary objects into whichever record system the practice runs (OEPR via TOMs-aware mapping; Cliniko via patient-form templates + chart notes). Treat OEPR as a future read/write target, not a competitor to be displaced.